



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : شيماء طارق حسني عبد الرزاق

العنوان : ش الباشا

الحي / المدينة : العمرانيه

المحافظة : الجيزه

تاريخ تسجيل الإستماره : 10-10-2026

رقم البطاقة : 29001272100247 رقم التليفون : 01152456415

العمر (تاريخ الميلاد) : 27-01-1990 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان

الوظيفة: -----
الوظيفة: -----
الوظيفة: -----

التوقيع: -----
التوقيع: -----
التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----

قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: شيماء طارق حسني عبد الرازق National ID: 29001272100247	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

شيماء
طارق حسنى عبد الرازق
٢ من عبدالله بن الباشا
العمرانية - الجيزة

١٩٩٠/٠٧/٢٧

٢٩٠٠١٢٧٢١٠٠٢٤٧

LK9489133



٢٠٢٥/١٠

٢٩٠٠١٢٧٢١٠٠٢٤٧

ربة منزل

متزوجة

مسلمة

أنثى

على عبدالستار راضى بيومى

البطاقة سارية حتى ٢٠٣٢/١٠/٢١



mm/dd/yyyy

MDT FORM

Name:	Shimaa Tarek Housni Abdelrazek	MRN:	2632
Age:	36 Y	ID:	
Menopausal Status:	Pre	Phone:	01118568519
NRS SCORE:		Marital Status:	M
BMI:		Family History:	Free
CO Morbidity:	On psychiatric disorder medication	Reason Of MDT:	Post NACT

Clinical Data:

BX: idc g2
ER 7/8 **PR** 8/8 **HER** 2+no **KI67**

Sonomammography:

06/06/2026: partial response with area of distortion
 2.3*.7 cm with thickened skin 3.5 mm
 Bilat nonspecific axillary LNs

Mets Workup:

History:

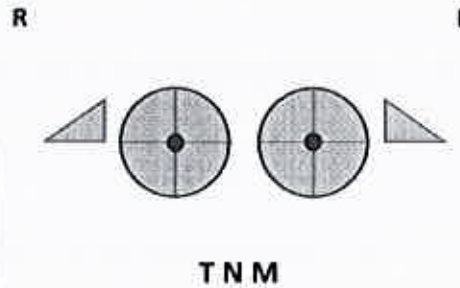
SM 6/10/2025
 right mass 11-1' o clock dimpling , skin thickening ,
 nipple retraction
 overlying skin 3 x1.6cm
 t4bn1m0
 CT cap : mediastinal LNs

Recommendations:

Operable; for Rt total mastectomy and SLNBx (pre N1
 now N0)

MDT Outcome & Plan

for Rt total mastectomy and SLNBx (pre N1 now N0)



SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هاني عزت	د. أحمد نجيب				

Name: شيماء طارق حسنى عبدالرازق			
MR. : 2632			
Detailed Medical Report			
		السيد	
تحية طيبة وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها			
The patient is 36 years old. There is no reported family history of breast cancer, and the only noted co-morbid condition is the use of antipsychotic medication. Immunohistochemistry demonstrates estrogen-receptor positivity of 7/8 and progesterone-receptor positivity of 8/8, with HER2 reported as negative. Imaging with sonomammography identifies a right-sided mass (RT MRM). No additional metastatic work-up findings are documented. The multidisciplinary team recommends postoperative radiotherapy (PORT) followed by systemic therapy with femara and zoladex, and a new CT scan of the chest, abdomen, and pelvis.			
تحريراً في ,			
التاريخ :		23-09-2026	
الوقت :			
الطبيب المعالج :			
يعتمد ,			

Patient Name : الأستاذة / شيماء طارق حسنى عبد الرازق :	Request No. : 13009978
Patient No. : 29001272100247	Request Date : 29/10/2025
Path No. : SR-1527/25	Verification Date : 11/11/2025
Sex : Female	Financial Category : جوسفان روسي - نفقة الدولة
Age : 35 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (6104-25). Together with, four referred slides including 1 H&E stained slide and 3 referred immunostained slides for (ER, PR & Her2).

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; SISH.

Microscopic Examination:

* Histological type: Invasive duct carcinoma.

* Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).

* DCIS: Absent.

* Angioinvasion: Absent.

* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).

- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).

- Her2: Intermediate incomplete membrane reaction in more than 10% of tumor cells (score 2+).

- SISH: Not amplified (Counted 3 times by 3 pathologists).

(Average Her2 copy No.: (4.5), Her2/CEP17 ratio: (1.9)).

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 2.

Verified by :

Dr.Duaa Mohamed



Patient Name	: الأستاذ / شيماء طارق حسنى عبد الرازق	Request No.	: 13009978
Patient No.	: 29001272100247	Request Date	: 29/10/2025
Path No.	: SR-1527/25	Verification Date	: 11/11/2025
Sex	: Female	Financial Category	: جوسف روسي - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

IPT:

- ER: Positive (score 8/8).
 - PR: Positive (score 8/8).
 - Her2: Equivocal (score 2+).
 - SISH: Not amplified, Group IV (Counted 3 times by 3 pathologists).
- (Average Her2 copy No.: (4.5), Her2/CEP17 ratio: (1.9)).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is

Verified by :

Dr.Duaa Mohamed

Printing Date : 12/11/2025 10:25:23

Page 2 of 3

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / شيماء طارق حسني عبد الرازق :	Request No. : 13009978
Patient No. : 29001272100247	Request Date : 29/10/2025
Path No. : SR-1527/25	Verification Date : 11/11/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 35 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

less than 4. (Recounting give the same result).

- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Prof. Dr. Maha Guimej.

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

Printing Date : 12/11/2025 10:25:23

Page 3 of 3

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

MRN: 10125272194
Patient Name: SHAIMAA TAREK HOSNY ABD EL RAZEK

D.O.B: 27/01/1990
IC No.: 29001272100247
Sex: Female
Location: External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2510210238	Collection Date	21/10/2025 01:22 PM
Accession No	IMM25-5526	Received Date	21/10/2025 01:23 PM
Sample Type	Paraffin Block	Doctor	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

6104/25

Clinical Data

Right breast cancer

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:

- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

Right

Diagnosis :

ER : positive, intermediate nuclear reaction in 70 % of tumor cells (7/8).

PR :positive, strong nuclear reaction in 90% of tumor cells (8/8).

Signature

Dr. Al Shima'a Zakaria, MD

Reported and electronically endorsed by

Mona Mohamed Ali Elabraq

MRN: 10125272194
Patient Name: SHAIMAA TAREK HOSNY ABD EL RAZEK

D.O.B: 27/01/1990
IC No.: 29001272100247
Sex: Female
Location: External Clinic

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2510210237
Accession No IMM25-5525
Sample Type Paraffin Block

Collection Date 21/10/2025 01:22 PM
Received Date 21/10/2025 01:22 PM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

6104/25

Clinical Data

Right breast cancer

Gross Picture :

Referred block submitted for HER 2 Immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $> 10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $> 10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, Intense in $> 10\%$ of tumor cells.

(Laterality) breast Cancer:

Right

Her2 Result

Equivocal [score 2+]. Recommended For SISH

Comment :

SISH is available upon request

Signature

Dr. Al Shimaa Zakaria, MD

Reported and electronically endorsed by
Mona Mohamed Ali Elabraq

MRN:	10125272194
Patient Name:	SHAMAA TAREK HOSNY ASD EL RAZEK
D.O.B:	27/01/1990
C No:	29001272100247
Sex:	Female
Location:	External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id 2510120207
Accession No SUR25-6101
Sample Type Tissue

Collection Date 12/10/2023 12:22 PM
Received Date 12/10/2023 12:22 PM
Doctor ASD ALLAH Mohamed Gamil

Clinical Information

Right breast 11-12 o'clock lesion, BIRADS 5, US guided biopsy.

Gross Picture

Multiple tissue cores, ranging in size from 0.4 cm to 1.4 cm long, Totally submitted.

Microscopic Picture

A case of invasive duct carcinoma, grade 2. No DCIS component nor lymphovascular emboli were detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnosis

Right breast 11-12 o'clock lesion, US guided biopsy:

Invasive Duct Carcinoma, grade 2.

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Marwa Mahmoud Abd Elkader, FRCR, IFCAP.

Reported and electronically endorsed by
Marwa Mahmoud Abdelkader Nada



Dear Prof. Dr.

Thank you for referring this case,

Date : 25/6/2024

Pateint Name: Shimaa Tarek Hosney



د/ منال إبراهيم
استاذة مساعد و استشاري الأجهزة التشخيصية

RIGHT SIDE REALTIME HIGH RESOLUTION
ULTRASONOGRAPHY REVEALED:-

- Small hypoechoic circumscribed focal lesion 7x6 mm seen at 1 Oclock, 3 cm from the nipple
- Normal parenchymal glandular and fatty tissue destribution is seen with prominent glandular tissue. It shows relatively abundant parenchyma in the upper and lower outer quadrants which is a normal finding.
- No evidence of malignant masses, no evidence of cysts or gross calcifications.
- Non spesific axillary lymphadenopathy with normal texture and preserved hilum.
- Normal skin, nipple and retroareolar area.

IMPRESSION:

- Small focal lesion as described , mostly fibroadenoma (BIRAD III), For follow up .

KINDLY ACCEPT MY BEST WISHES,
Dr/ Manal Ibrahim, MD

Patient Name: Shaimaa Tarek Hussein
Exam : CT CAP+C (CT Chest Abd. Pelvis)
Medical ID: 3096/2025

Date : 12/11/2025
Age : 35 Year(s)
Pat ID: 132889

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS

REVEALED: -

Reported case with right breast cancer

1. Chest:

- Right breast speculated mass 4 x 2 cm .
- Left axillary level I and II pathologically enlarged lymph nodes the largest measures 21 x 11 mm.
- No sizable hilar or mediastinal lymph nodes, the largest seen at right hilar region measures 21 x 22 mm..
- No sizable pulmonary nodules or masses.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Right breast mass with ipsilateral axillary lymph nodes as described.***
- ***No pulmonary , hepatic or bony metastatic deposits.***

DR.reham kheder

9/14/26, 4:32 PM

code:09 – Name_A MDT FORM – Name_E MDT FORM

جمهورية
مصر العربية
وزارة الصحةمبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة العامة
للتنظيم
الاسكاني

09/14/2026

MDT FORM

Name:	Shimaa Tarek Housni Abdelrazek	MRN:	2632
Age:	37 Y	ID:	29001272100247
Menopausal Sta		Phone: *	0118568519
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	-VE
Gender		Reason Of MDT:	
CO Morbidity:	ANTIPSYCHOTIC	POST NACT , POST OP	
Surgical History: *	2 CS RT MRM	ECOG Performance Status	0 0 1 0 2 0 3 0 4

Clinical Data:

R

L

BX:

ER: 7/8

PR: 8/8

HER2: NOT

KI67:

History:

Sonomammography:

RT MRM

Biomarker/Molecular

RESIDUAL IDC G2

5% LOW GRADE DCIS

FREE MARGINS

LN 4/10

PPR

Mets Workup:

Clinical Staging:

Recommendations:

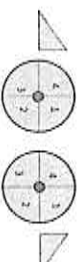
Pathological Staging:

MDT Outcome & Plan

PORT

FEMARA + ZOLADEX

NEW CT CAP



T N M

Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY



Patient Name	شيماء طارق حنى	Case No.	345490
Sex/Age	Female 36 Y	Patient No.	25424
Registered Date	02/8/2026 10:19:00 AM	Referred By Dr.	Surgery
Validation Date	30/8/2026 09:55:16 AM	Branch	Gustave Roussy
Reporting Date	10/9/2026 12:39:50 PM	Pathology No.	2417/26

Pathology Report

Clinical Data: Right breast cancer (post-NACT).

Nature of the specimen: Right simple mastectomy, Sentinel lymph nodes biopsy & Axillary clearance.

Gross Examination:

- * Right simple mastectomy specimen measured 25x20x10 cm, with covering skin ellipse 25x20 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site measured 3.5x2.0x2.0 cm, located 3.5 cm from the least deep margin.
- * Sentinel lymph nodes: Multiple fibrofatty tissue piece measured 5.0x5.0 cm, entangling 4 lymph nodes, the largest one measured 1.0x1.0 cm.
- * Axillary lymph nodes: Multiple fibrofatty tissue piece measured 8.0x8.0 cm, entangling 4 lymph nodes, the largest one measured 1.5x1.5 cm.

* Block designation "9 Blocks":

1:3- Mass,, 4- Deep margin,, 5- Nipple &Areola,, 6- SLNs,, 7:9- ALNs.

- Gross was done by: Dr. Aya Ezzat.

** Frozen (Intra-operative) results:

- SLNs: Positive viable metastatic deposits in two out of four lymph nodes (2/4).
- Frozen sections examined by: Prof. Dr. Gamal Abd Elaty.

Microscopic examination:

- **Histological type:** Residual invasive duct carcinoma.
- **Cellularity:** 50%.
- **Grade:** 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** 5% Low grade DCIS (Solid& Cribriform patterns).
- **Lympho-vascular emboli:** Present.
- **Microcalcification:** Present.
- **Perineural invasion:** Absent.
- **Nipple and areola:** Free.
- **Resection Margins:** Free deep margin.
- **Sentinel lymph node involvement:** Positive viable metastatic deposits in two out of four lymph nodes (2/4).





Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي - قصر زنتونال - قصر عميل البقاولجي



Patient Name	شيماء طارق حسني	Case No.	345490
Sex/Age	Female 36 Y	Patient No.	25424
Registered Date	02/8/2026 10:19:00 AM	Referred By Dr.	Surgery
Validation Date	30/8/2026 09:55:16 AM	Branch	Gustave Roussy
Reporting Date	10/9/2026 12:39:50 PM	Pathology No.	2417/26

- Axillary lymph node involvement: Positive viable metastatic deposits in two out of six lymph nodes (2/6).
- Total lymph nodes status: Positive viable metastatic deposits in four out of ten lymph nodes (4/10) (The largest nodal metastasis measuring 0.6 cm, with extra-nodal extension).
- Histologic response to therapy: Pathological partial response to therapy.

Diagnosis:

Right breast, (post-NACT), simple mastectomy, Sentinel lymph nodes biopsy& Axillary clearance:

- Residual invasive duct carcinoma, grade 2. Cellularity (50%).
- 5% Low grade ductal carcinoma in situ (Solid & Cribriform patterns).
- Nipple & areola: Free.
- Deep margin: Free.
- Sentinel lymph nodes: Positive viable metastatic deposits in two out of four lymph nodes (2/4).
- Axillary lymph nodes: Positive viable metastatic deposits in two out of six lymph nodes (2/6).
- Total lymph nodes status: Positive viable metastatic deposits in four out of ten lymph nodes (4/10) (The largest nodal metastasis measuring 0.6 cm) with extra-nodal extension.
- Pathological partial response to therapy.
- Stage: ypT2, ypN2a.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



خريطة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - قصر القبة - القاهرة - مصر
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